To: Governor of California
From: Khairy Barnes
Date: 10/04/2026
Subject: Implementation of Proposition 36 in Northern California

Executive Summary

In response to the opioid epidemic's rise in addiction and overdose deaths, California enacted Proposition 36, known as the “Substance Abuse and Crime Prevention Act,” to provide rehabilitation services to nonviolent individuals struggling with addiction. However, unforeseen circumstances led to major problems. The issue wasn't just that Proposition 36 was imposed from the top; it was also that California imposed a statewide obligation while withdrawing the resources needed to carry it out. Northern California counties responded efficiently by establishing a regional, bottom-up implementation network. However, this decentralization introduced new challenges related to funding, provider incentives, oversight, and accountability. As a result, California should adopt a hybrid approach that blends state resources and performance standards with greater local flexibility.

Top-Down Implementation and Its Defects

Based on Sabatier’s 1986 article “Top-down and Bottom-up Approaches to Implementation Research: A Critical Analysis and Suggested Synthesis,” Proposition 36 fits a top-down approach because implementation begins with an authoritative state policy decision and asks whether implementing officials and target groups carry out the objectives established by that decision. Sabatier also identifies several conditions that affect the likelihood of successful implementation, including clear, consistent objectives; an adequate causal theory; sufficient financial resources; coordination among implementing institutions; appropriate decision rules; and committed implementing officials. The state of California’s main objective was to help individuals suffering from drug addiction. The plan was to provide people statewide who had drug offenses with accessible and effective treatment instead of incarceration, as it would reduce their substance dependency. The state initially directed $120 million per year toward the program; however, as economic hardships and the opioid crisis occurred, funding for the program dwindled while demand for the services increased. This led to the creation of the NCRC, as officials believed delegating decision-making to a regional consortium would improve coordination through pooled expertise, unified grant-seeking, and increased regional monitoring. Judges in California state courts strictly controlled access, allowing offenders to become probationers and become eligible for rehabilitation. However, frontline treatment providers had more avenues, with relatively wide discretion to provide services after NCRC caseworkers referred a client to them. Sabatier’s definition of the top-down approach applies to Proposition 36, as it has clear objectives, an adequate theory, and effective coordination through delegation to regional offices. The financial resources were insufficient, and the approach shifted; however, it always had a clear mandate and a causal theory, along with committed implementing officials and effective coordination among implementing institutions; thus, Proposition 36 began with a distinctly top-down structure, although changing economic and social conditions eventually forced its implementation to incorporate increasingly bottom-up features.

Funding appears to be the biggest issue with the top-down approach. As funding dissipated, the mandate remained, creating a mismatch between rising demand from the opioid epidemic and the reduced capacity of local offices to meet recipients' needs. Sabatier defines successful implementation as depending not only on clear objectives but also on resources, implementation capacity, political and economic conditions, and implementer commitment, none of which Proposition 36 accounted for. Furthermore, Lindquist and Wanna, in their 2015 article, "Is Implementation Only About Policy Execution?: Advice for Public Sector Leaders from the Literature," argue that implementation requires a clear mandate and sufficient capability, while allowing for adjustment when conditions change. They also describe policies whose effectiveness deteriorates as constituency support erodes, gradually weakening the original policy. Proposition 36 followed a similar trajectory: state funding declined from its initial level before disappearing entirely in 2010, even though the mandate remained. This, combined with increased demand for support due to the opioid crisis, created issues that required the program to adjust its approach. The top-down approach was no longer effective because it did not account for economic conditions and resource availability. Top-down systems can fail when the people designing or directing policy are disconnected from those implementing it. The interview with Jennifer Polker at NPR described a similar situation: In their case, adding more staff made their situation worse; information wasn't traveling effectively from implementers upward. The people closest to implementation understood some of these problems, but leadership was reluctant to communicate them upward because of political optics. Both this case and Proposition 36 show that top-down implementation can become ineffective when policymakers prescribe solutions without proper feedback from the actors delivering the policy.

Shift Toward Bottom-Up Implementation

The defects of the top-down approach became apparent by 2010, when funding was fully cut but the mandate remained. Implementation was no longer determined primarily by the original state design; local actors built the delivery system in response to local conditions, with Humboldt, Trinity, Butte, Mendocino, and Shasta County cooperating to form the NCRC. This echoes Lindquist and Wanna mention of 'interorganizational policy implementation,' explored by O'Toole and Montjoy (1984). This method examines various types of interdependence among organizations that arise from the policy's nature, authority distribution, and task demands. It emphasizes bottom-up strategies that focus on implementing agencies' roles and the importance of collaboration among agencies, frontline staff, and service providers. These bottom-up strategies rely on a patchwork of payment sources. Poorer probationers eligible for Prop 36 drug treatment could receive coverage through Medi-Cal or Covered California. The NCRC also contracted with private, non-profit, and for-profit providers to deliver front-line treatments. This most closely aligns with Lindquist and Wanna's mention of "Exchange and bargaining" by Stoker (1989). This framework balances top-down concerns, such as the state of California seeking compliance amid the opioid epidemic and state funding cuts, with bottom-up factors: fractured authority, private-sector bids through a "request for proposals" process, and conflict among middle and frontline organizations and clients. In this model, implementation is framed as an exchange process in which different actors can add value, and it does not assume that all politics is completed in the design phase. It also helps deal with unforeseen circumstances. Implementation of Proposition 36 increasingly depended on a network of local governments, public programs, nonprofit organizations, and private providers acting as problem-solvers. This

model works because it accounts for a dynamic, evolutionary approach from the start. For Proposition 36, a centralized consortium like the NCRC made the exchange-and-bargain approach effective, as it could offer higher total caseloads and a larger contiguous service area than any one county alone. In theory, this would lead to more frequent and higher-quality bids from potential providers.

Effectiveness and Limitations of the Bottom-Up Approach

In practice, the bottom-up shift had its costs and benefits. By diversifying its funding sources, the NCRC had multiple ways to sustain itself after it lost government funding. Whether through wealthier patients paying out of pocket, Medi-Cal coverage, or policies obtained through the California health insurance marketplace, NCRC was able to at least continue the service, even as it still struggled to fund growing caseloads of probationers. By maintaining this funding plan for long enough, they were able to hold out for other groups to step in and provide aid. The California Health Care Foundation gave the NCRC a $15 million grant to establish and expand drug treatment service options for Proposition 36 probationers. This allowed the group to fund drug treatment services for those who had no ability to pay, as well as reduce wait times and waiting lists for treatment. Additionally, by cooperating with private contractors, they served 700 program clients each year, split among different contractors, placing clients with a provider based on proximity to the nearest treatment facility, which increased the program's overall capacity. While this solved the flexibility problem, it created an accountability problem. To meet demand for treatment services, the NCRC contracted with a newer, less-proven for-profit organization, Treatment Strategies Inc.; one major concern was cream skimming, which restricted services to clients considered easier to serve and neglected more challenging cases. This impacts the program’s overall effectiveness. Despite extra funding, the program still had a long annual waiting list of 200-300 clients needing assistance. Furthermore, the quality of service varied depending on how difficult each case was. Lastly, little to no information was available about the system's effectiveness; neither the private organizations nor the NCRC appeared to provide any data on their services' effectiveness.

Recommendation: A Hybrid Implementation Strategy

I recommend that California adopt a hybrid implementation strategy that restores stable state funding, establishes statewide accountability standards, and requires systematic performance measurement while preserving county flexibility over regional organization, provider selection, and service delivery. I chose these recommendations because this case demonstrates that pure top-down control lacks adaptability, while bottom-up implementation without adequate state resources and oversight lacks capacity/accountability. Long and Franklin highlight this in their 2004 analysis, “The Paradox of Implementing the Government Performance and Results Act: Top-Down Direction for Bottom-Up Implementation.” In this case, the GPRA encountered a similar core problem as California: it relied on procedural controls instead of tackling the underlying structural issues. The GPRA was paradoxical: although it set a top-down, uniform policy direction, it required a bottom-up implementation approach. As with the agencies the GPRA directed, the NCRC found it hard to gauge success across multiple providers, since private organizations can use funds in many ways, intended and unintended. Even if the State of California determines that a provider is not fully meeting its contractual obligations, stronger

monitoring alone may accomplish little if funding and incentives remain disconnected from performance. For this program to succeed, centralized and decentralized implementation should not be mutually exclusive; implementation can combine higher-level constraints with lower-level interpretation and discretion. As Long and Franklin show, imposing procedural requirements from above does not necessarily address the underlying structural conditions affecting implementation.